

Blistering Cutaneous Reaction Temporally Associated with a Discoloured Batch of Fevrolix Tablets: A Case Report

A. Menon MD (1) | (1) Dept. of Dermatology, Test City Hospital

Abstract: A 45-year-old man developed a severe blistering skin rash within 24 hours of taking Fevrolix tablets from a batch noted by the dispensing pharmacist to be visually discoloured compared to prior batches. We describe the clinical course and discuss the possibility of a manufacturing quality issue contributing to, or coinciding with, an adverse cutaneous reaction. (Fictional case, created for software testing purposes.)

Case Presentation

A 45-year-old man with no prior history of drug allergy took a single dose of Fevrolix tablets for fever. Within 24 hours he developed an erythematous rash with early blistering, most prominent on the trunk and forearms. He sought dermatology consultation; the medication was discontinued immediately and he was managed conservatively with topical corticosteroids and oral antihistamines, with gradual improvement over the following week without hospitalization. Notably, the dispensing pharmacist reported that the remaining tablets in the strip appeared yellowish rather than the expected white colour, raising the possibility of a batch-level manufacturing irregularity.

Discussion

Cutaneous drug reactions range from mild exanthems to severe blistering conditions and require prompt recognition. In this case, the temporal association with a visually atypical batch raises two non-exclusive possibilities: an idiosyncratic immune-mediated drug reaction, or an excipient/formulation irregularity introducing a novel sensitizing component. Batch-level quality investigation was recommended in parallel with adverse event reporting.

Conclusion

This case underscores the value of cross-referencing adverse event reports with product quality signals, since discoloration or other visible batch defects can be a relevant contributing factor in reaction causality assessment.

References

1. Roujeau JC, Stern RS. Severe adverse cutaneous reactions to drugs. *N Engl J Med.* 1994;331(19):1272-1285.
2. Test Pharmacopoeia Committee. Visual inspection standards for solid oral dosage forms. 2020.